

SUBJECTIVE:

- Patient presents with acute chest pain, stating "I have a chest pain" and clarifying its location as "in my chest."
- Further history is crucial to fully characterize the pain, including its onset, duration, quality (e.g., sharp, dull, pressure, squeezing), severity on a 1-10 scale, radiation (e.g., to arm, neck, jaw, back), and any aggravating or alleviating factors (e.g., activity, rest, position, food intake).
- Associated symptoms must be elicited, such as shortness of breath, palpitations, dizziness, syncope, nausea, vomiting, diaphoresis, cough, or fever.
- Review of relevant past medical history is essential, including known cardiac conditions (e.g., CAD, hypertension, diabetes), pulmonary diseases, gastrointestinal issues (e.g., GERD), and any recent trauma.
- Social history should include tobacco use, alcohol consumption, and illicit drug use. Family history of cardiac disease is also pertinent.

OBJECTIVE:

- General: Assess for signs of distress, pallor, or diaphoresis.
- Cardiovascular: Auscultate heart for rate, rhythm, murmurs, rubs, or gallops. Evaluate for jugular venous distention (JVD) and peripheral edema.
- Respiratory: Assess respiratory rate and effort. Auscultate lung fields for clear breath sounds, wheezes, rales, or rhonchi.
- Abdominal: Palpate for epigastric or right upper quadrant tenderness, guarding, or rebound.
- Musculoskeletal/Skin: Palpate the chest wall for tenderness. Inspect the skin for rashes or lesions, particularly vesicular eruptions.
- Vital signs: Vital signs are currently unavailable but are critical for assessment and include blood pressure (bilateral), heart rate, respiratory rate, temperature, and oxygen saturation.

ASSESSMENT:

- Primary diagnosis: Given the chief complaint of chest pain, an acute coronary syndrome (ACS) – including unstable angina or myocardial infarction – must be considered as a life-threatening diagnosis until proven otherwise.
- Differential diagnoses:
 - Cardiac: Pericarditis, Myocarditis, Aortic Dissection, Prinzmetal's Angina.
 - Pulmonary: Pulmonary Embolism, Pneumothorax, Pneumonia, Pleurisy.
 - Gastrointestinal: Gastroesophageal Reflux Disease (GERD), Esophageal Spasm, Peptic Ulcer Disease, Cholecystitis.
 - Musculoskeletal: Costochondritis, Rib fracture, Muscle strain, Herpes Zoster.
 - Psychiatric: Panic Attack, Anxiety.

PLAN:

- Tests:
 - Immediate: Electrocardiogram (ECG) to assess for ischemic changes, serial cardiac enzyme levels (Troponin I/T) to rule out myocardial injury, complete blood count (CBC), basic metabolic panel (BMP), and Chest X-ray (CXR) to evaluate for pulmonary or mediastinal causes.
 - Further consideration: D-dimer if pulmonary embolism is suspected, CT Angiography of the chest for suspected pulmonary embolism or aortic dissection, upper endoscopy if gastrointestinal etiology is strongly suspected.
- Medications:
 - If ACS is suspected: Aspirin 325 mg chewable, sublingual Nitroglycerin (if blood pressure permits), supplemental Oxygen if hypoxic, IV access for pain control (e.g., Morphine) and antiemetics if needed.
 - Other: Proton pump inhibitors (PPI) for suspected GERD, NSAIDs or acetaminophen for musculoskeletal pain, antibiotics for suspected infection.
- Patient advice: Advise the patient to remain calm, avoid any strenuous activity, and report any changes in symptoms immediately. Emphasize the importance of following medical recommendations given the serious nature of chest pain.
- Follow-up instructions: Urgent evaluation in an emergency department is strongly recommended for any new onset or worsening chest pain. Further follow-up with a primary care physician or specialist (e.g., cardiologist, pulmonologist, gastroenterologist) will be arranged based on the definitive diagnosis.